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COMPANY DEEP-DIVE

Founding Sales Executive at Thoughtful AI Believes AI Agents Can Streamline Revenue Cycle Management Processes

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18 Dec 2024	19 Sep 2024	Consultant	Investor-Led (Buy-Side)	DENTALXCHANGE

SMART SUMMARY

Summary

The conversation covered the application of AI agents in automating revenue cycle management processes for non-hospital healthcare providers, including technical capabilities, integration with existing systems, pricing models, and buyer receptivity. **The discussion also addressed the limitations of current automated software and the specific workflows where AI agents provide the most value**.

EXPERT BACKGROUND

- Founding sales executive at an AI solutions provider focused on building and selling custom AI agents for revenue cycle management (RCM) processes, with direct experience selling to CFOs, VPs of RCM, and CEOs at non-hospital healthcare providers across multiple specialties.

EXPERT SENTIMENT

- The expert is optimistic about the adoption and effectiveness of AI agents in automating complex RCM workflows, particularly in non-hospital healthcare settings.
- They believe that while **full automation of certain manual processes is possible in the future, significant technical and investment barriers** remain, especially in complex specialties like behavioral health and dental.
- **The expert is confident in the strong and growing receptivity of the target buyer universe to AI agent solutions, citing high inbound interest and positive cost and control dynamics compared to traditional outsourcing**.

TAKEAWAYS

- AI agents are designed to operate within existing EHR and claims software, automating manual RCM tasks such as prior authorization, eligibility verification, and claims management by mimicking human workflows and leveraging technologies like RPA, OCR, and GenAI.
- Current automated software solutions often fail to handle complex exceptions and edge cases, requiring human intervention, whereas AI agents can address these gaps more effectively.
- The main buyers are non-hospital, rapidly growing provider groups (often PE-backed) with large RCM teams, and pricing is based on the number and complexity of agents rather than transaction volume.
- AI agents typically reduce the manual workload for RCM teams, with industry benchmarks for automation ranging from 50% to 80% of transaction volume, depending on process complexity.
- There is **strong buyer interest in AI agent solutions due to their transparency**, control, and cost advantages over outsourcing, with additional potential in pharma, med device, and infusion sectors.

AlphaSense Assistant is AI Generated

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Expert Bio

Founding Sales Executive at Thoughtful AI. Founding Sales Executive at Thoughtful AI. The expert is responsible for running the full sales cycle from discovery through contracting and ongoing strategic relationship/account management. The primary buyers/stakeholders are CFOs and VPs of RCM at non-hospital healthcare providers. Prior to Thoughtful AI, the expert was a Strategic Account Executive at Eightfold, leaving April 2024. The expert was responsible for managing customer-partner relationships with strategic partners who have purchased and co-developed Eightfold-built solutions, with a focus on account revenue growth and joint GTM strategy and execution. This was a dual role with both account management between Eightfold and their customer (revenue retention and growth) and strategic partnership to define GTM strategy and execution to grow customer-partner's revenue from selling Eightfold-built solutions. The expert can speak to the SaaS, AI, and data space. The expert excels at relationship and account management, people development, and team leadership with professional services, tech, healthcare, and public sector experience across functions.

Interview Transcript

Client 00:00:00

Hey there. Thanks so much for taking the time. Just to provide a bit of context, we are just doing some general research into kind of GenAI and various RCM applications, more so understanding kind of how various companies are using technology and some of the solutions out there. So kind of given your experience with Thoughtful it would be helpful to chat today.

Expert 00:00:21

Sure. Yes, thanks for the context, and happy to help. Just something exciting for you all to look into, Thoughtful just published on the website today. PAULA, which is the newest AI agent specifically for prior authorization. So that's on the website, and I think there should be some press coming out around it on LinkedIn and other places. So as you're doing more research about AI and prior authorization, just be aware of that.

Client 00:00:46

Awesome. Well, sounds like it was very timely.

Expert 00:00:49

Yes, exactly.

Client 00:00:50

Great. So maybe just to help level set the conversation and see better direct questions. Could you just provide a brief overview of kind of your role and what you do at Thoughtful?

Expert 00:01:00

Yes, sure. So I'm a founding sales executive at Thoughtful AI. And what I do specifically is I work to generate sales leads and actually sell deals discovery through, proposal through contracting with CFOs and VPs of RCM as well as CEOs at nonhospital health care providers across specialties.

So ***Thoughtful AI*** can build custom ***AI agents*** for a number of ***revenue cycle*** management processes, prior authorization being focus of conversation today. But again, we work with any number of specialties, being behavioral, dental, orthopt, vision, podiatry, vascular, et cetera, and we build a custom AI agent that actually does the work of revenue cycle management employee like a digital worker. And my job is direct sales from discovery through contracting.

Client 00:01:46

Got it. And just to kind of understand the solution -- when you say an AI agent, like what service are you actually providing?

Expert 00:01:53

Yes. So a lot of players in this space may have like a platform or a software solution that integrates with an EHR and EMR or practice management system. And it's like that's just something through which RCM employees submit prior authorizations.

Thoughtful actually builds an AI agent that if you think about it at the most basic level, functions kind of like an RPA bot. It picks up a work queue of prior authorization needs from patient registration, patient appointments, and it actually navigates through the prior authorization process, doing the work that a human would be doing.

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So collecting documentation, staying abreast of payer policies and then actually taking whatever is required if there's a prior auth for a given service or set of services, filing that information directly into payer portals and then monitoring the prior authorization process.

And then like handling resolution and denials using like GenAI to generate appeal letters or initial letters based on clinical notes or things that are uploaded to the systems that the AI agent is operating. So don't think of it as a new system as much as like service as a software in the form of a digital worker that is like an RPA bot, but involves additional AI technologies beyond RPA.

Client 00:03:11

Got it. And so like the bot is working in customers' existing kind of EHR and claims software tech stack?

Expert 00:03:17

Exactly. We are not a new software. We are operating and optimizing the systems that a provider has currently.

Client 00:03:24

Got it. Are **you worried about a customer switching** that's more automated or can do a lot more of the kind of manual task?

Expert 00:03:31

Worried, no. We get this question a lot. I mean, think about it like this. Like even if, for example, an EHR or a clearinghouse automates certain things, a provider still has humans who have to monitor that process and who have to do some form of manual work, whether that's upfront or in the middle or back end, an AI agent can still take the place, if you will, of a human or a group of humans that are doing that, even if there is automation built into the process itself.

Client 00:04:00

What are some of those manual things that you think software won't be able to automate and what an AI bot can handle?

Expert 00:04:07

Yes. So I mean, I think, honestly, the biggest things that we see are at sort of the handoffs between clinical staff who are perhaps generating a letter or some documentation, if you will, to explain why a given procedure is needed. That handoff from clinical to administrative staff is often clunky and not necessarily handled in an automated way.

And then I would likewise say that we haven't yet seen and it doesn't mean it doesn't exist. It just may not be public information as far as I've seen from others, really a good handle on the ability for a system to automatically collect all of the documentation in the form of whether they are preapproval letters.

Or the form of previous medical records or other elements that get submitted in a prior authorization packet or that are included in like a prior authorization appeal letter. It's those elements sort of doing the things that get documentation into a place where it can be automatically submitted or followed up on that humans are still doing, at least in my experience.

Client 00:05:11

You haven't seen anyone do it, but do you think it could be kind of automated away rendering like the AI agent in a use case kind of.

Expert 00:05:19

I mean someday possibly. I'll never say never. I haven't seen that done yet. And I think it will be a long time, especially in certain specialties like dental or orthodontia or even like behavioral health before that happens, but never say never.

Client 00:05:33

Why those specialties specifically?

Expert 00:05:35

Yes. Well, from my perspective, **I have not yet seen a lot of very strong investment in automated solutions in those specialties** that can handle the vagaries and nuances of the large number and, let's say, complexity of services delivered in the behavioral and mental health space.

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And likewise, the complexity of different procedures and specific documentation all the way down to like specific photographs of specific teeth that are needed for some of the dental and orthodontia prior authorizations needed before claims can be generated. But again, that's just my experience.

Client 00:06:11

Got it. And then just a clarification on what you said, like the handoff between the admin staff being kind of something that you haven't seen as fairly automated. What do you mean by handoff? Like what are the actual work that's happening there?

Expert 00:06:26

Yes. So in certain instances, let's take behavioral health, for example, before a patient can be seen for certain types of mental health-related services, a clinician actually has to -- especially if you're thinking like community like out in the field-based work, has to actually take notes, upload those notes to somewhere and then have those notes turned into a prior authorization letter.

Depending on the workflow, sometimes the clinicians have to generate the letter themselves, **which places a lot of onus on them** and it doesn't seem to have been done well yet in my experience or more commonly from what I've seen, the notes are taken and a human RCM employee is translating those notes into a letter and then putting that letter into the system and submitting it.

To your earlier question, that could in some way -- it could in some way be automated. Eleos, for example, is a company that's trying to use ambient listening to automate some of the clinician notes, et cetera. But thus far, I have not seen an effective automated way that clinician submits notes, notes are automatically generated into a letter and then that letter is automatically submitted from a nonclinical system into, let's say, the claims or billing systems.

Client 00:07:42

I'm sorry, just a clarification on the behavioral health example. What kind of like services are you talking about here? Because I thought billing like office visits.

Expert 00:07:52

Yes. So if you think about behavioral and mental health as broadly defined, there are a lot of community-based behavioral health services that are delivered where you're actually like clinicians or trained behavioral health specialists are going out to meet people, whether in their homes or if you're talking about populations that may be unhoused, meeting them just physically where they are at a given time.

And so there's not a quick determination like of someone who has filled out paperwork, let's say, before they come into an office and know exactly what services are going to be delivered. And this may be a niche use case, but it's one I've encountered a number of times.

Client 00:08:33

Got it. Yes, that's helpful. And this discussion about like what you don't think can be automated by or like how you expect it can be automated by software, was a little bit more siloed to the prior auth example. But I'm curious, just a broader like RCM, particularly on the claims and billing side, if there's anything else that you would flag that like pretty much you think an AI agent will only be able to do for a while versus kind of automated software.

Expert 00:09:02

Yes. So for eligibility verification, in particular, the biggest pain point that we encounter, and I'm sure any solutions provider in this space does because it's written about publicly, to verify eligibility, like there are certainly plenty of automated services that try to do this.

Oftentimes, what those automated services do is they take patient registration. They check whatever patient information is in the EHR, and they just do a quick check if they have an integration with a payer portal, for example, for eligibility verification.

And they essentially return like a green check or a red X, do they have these? What's not done and **what is really seeming to be missing and challenging for those providers** to provide is the ability to pull specific information from potentially changing insurance documentation related to a given patient's health plan or health policy back into the EHR and actually put it in specific fields, co-pay, coinsurance, deductible, et cetera, beyond just does this patient have the insurance we thought they have.

And so you have humans who are continuing to do that or having to follow up because **the information is inaccurate**. A software that can truly do that in an automated fashion, that sure, that may come about someday, but that's going to require additional AI capabilities that don't currently exist in a lot of those automated softwares.

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And that's where the AI agent concept and the ability of the AI agents to actually navigate through those payer systems and full specific information and put it into specific places has really had a huge selling point at this point.

Client 00:10:43

And just a clarification. You say like those automated software coming today don't have the AI capabilities. But do those AI capabilities like exist from a technical feasibility perspective, and they just haven't been applied? Or are they like have yet to be developed to what you need?

Expert 00:11:00

Yes, I think they exist. I mean, I'll put it this way, **one of the major technical challenges in this** is including different types of AI capabilities, let's say, for example, RAG, retrieval augmented generation, at a point where, let's say, an automated software runs into an exception, it doesn't know the answer, it will just return no in the current state.

And **I think that there's just been a lack of investment in development** mainly because it's very expensive. And so a lot of these automated software providers for eligibility verification or claims, they've built something that works 50% to 70% of the time, but **that's not good enough for providers**.

Client 00:11:41

When you say it's very expensive, like what makes it expensive and what would need to be done incrementally?

Expert 00:11:47

Yes. So if you think about it from the front of the expense, if you're developing this from scratch, obviously, the R&D cost in the form of software engineers, the engineering manpower that it would take to build something, the compute infrastructure in terms of cloud-based costs and things like that, if you're building from scratch, if you've got something that you already have, you already have an automated software, let's say, again, a lot of it is R&D in the form of engineers, the cost of them, the time it takes to add something to an existing system.

And then the time it takes to roll that out. R&D in the space, frankly, especially if you already have a product and adding something new to your, let's say, the way something is technologically architected is just very expensive from what I've seen and what others have told us who've tried to build similar things.

Client 00:12:41

Got it. And then in terms of maybe more like the claims scrubbing submission the RCM chain, like what are the tasks that you think an AI agent can do there, but that can't really -- are barriers to have like an automated software solution today?

Expert 00:12:56

Yes. I think in the end a lot of this comes back if we want to categorize it. It comes back to the edge cases or the exceptions that aren't just a straightforward upload this documentation and submit it through the automated system. If there's anything related to like denials management, additional information is needed.

So the specific example I see a lot across specialties is **a claim is denied because either the medical documentation wasn't submitted correctly or incorrect** documentation was submitted. So maybe the automated claim software submitted something because it saw there was an attachment there and so submitted the claim, but it wasn't the right information.

That is something that currently automated software really can't handle, and **it would need to include additional capabilities of being able to search and follow particular rule sets to find specific missing documentation or to generate a denials appeal letter** that those automated softwares don't have. That's where humans are still heavily in the loop and where an AI agent in today's day and age is handling sort of like the areas where even automated software, claims submission software doesn't work.

Client 00:14:07

Got it. And maybe dumb questions, but like why don't those software companies today kind of build their own AI agents?

Expert 00:14:14

Yes. It's not a dumb question at all. Well, several reasons. Frankly the concept of an AI agent beyond just straightforward like RPA bots is relatively new. So that's number one. I mean all sorts of tech gurus talk about that publicly. That's number one. Number two, a lot of these

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claims, these automated softwares, et cetera from what I can see, **they're making money** currently, and they don't know what's coming next.

Now some of the leadership at these companies may, and so they're taking steps to include AI agent-like capabilities to operate their systems on behalf of their customers, that's certainly something that you can read about in the press. But I would just say that like those systems were created for a particular use case.

They have probably worked okay better than the totally manual work that was happening before, but **they're being disrupted by the next wave of burgeoning technology** that maybe they will adapt to, maybe an AI agent will be brought in to operate those automated systems on behalf of end customer providers or maybe that will be developed into those automated softwares. But I think really the novelty of the technologies, the cost and frankly, just a little bit of inertia are all combinations for why you haven't seen more of that yet.

Client 00:15:31

We're saying like automated software like RCM software companies kind of broadly, but are there any names that come to mind?

Expert 00:15:39

Yes, absolutely. So I mean, if you think about like eligibility verification, Experian, it comes up. We hear that relatively frequently. I mean the clearinghouses like Waystar, for instance, are building in automation for both eligibility checks and then for claims submission. Those are certainly some of the biggest names that we come across in the space where we play.

At a space where Thoughtful AI doesn't play, the Epic space, Epic is always trying to build on more automation for the large hospital systems that are on the Epic EHR. So those are some names we come across. And then I guess in the dental space specifically, dental exchange is a common name we hear in both dental and orthodontics.

They're trying to build in some automation. If you're interested, and you don't have to answer this question for me, but I'll just give you the name, like if you guys are interested in the dental space at all, check out dental robots, for example. They're not an automated software as much as an RPA bot provider, but those are all names that are out there that we come across relatively frequently. And if you did a Google search, you'd find any one of those.

Client 00:16:46

Got it. When you come across Adonis on the claims side?

Expert 00:16:50

Yes, we have. Yes. So my read on Adonis, and this is one person's read, is Adonis is a system that, I don't know, I can't tell you how effective it is, but we certainly hear about it. It's a system that can help in the RCM space with some of the automation, they pitch themselves with a lot of analytics. Their website is all about analytics.

Interestingly enough, if you look at the services that Adonis provides and some of the roles that they're hiring for, it's pretty clear that they use outsourced teams to do some of the things they may claim they're automating on behalf of their customers.

Again, that's just my interpretation. But if you look at all of those data points, you all can come to your own conclusion. But we certainly come across Adonis. It's a hot name in the space. They just raised, I think it was a Series B. And certainly wouldn't say anything negative even if I heard it, but I haven't heard anything negative, just being honest.

Client 00:17:48

Got it. What are the software systems that your AI agents most commonly kind of sit on top of and work with them? Like is there like a tech stack that you.

Expert 00:17:56

Yes. I would say if you go to the Thoughtful website, I don't have the exact link, but it's public. You can find a couple of quick clicks. You could find many of the systems listed there. But I'll tell you which of those that are on that list I frequently come across. So we see Athenahealth a lot in the space where we play. We see NextGen a lot. We see DentalXChange, Waystar. We previously saw Change Healthcare a lot prior to the data breach.

The Waystar, big one, a very big one that we come across a lot. Obviously, like I said, we are an Epic, this is publicly available information. We are an Epic-certified vendor, but we don't generally work with large hospital systems. So we come across Epic, but we don't generally build AI agents that operate that system. So those are a few.

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Then again, DentalXChange in the dental space in the -- what is the one in behavioral health. I'm blanking on the name of the one we see frequently in behavioral health. Netsmart in behavioral health and, InfuseFlow is one we come across pretty regularly.

And then, of course, who's who of payer portals. The BCBS, UHC of the world, Delta Dental from a payer side. So the AI agents can log into and operate any of the systems I've just mentioned. And frankly, they can operate almost anything, but those are the ones that come across very regularly.

Client 00:19:14

Got it. And it seems like you rattled off mostly each or some clearinghouses and some like commonly encounter any like point solution like RCM software that your agents are navigating for the customer?

Expert 00:19:27

We come across Experity in the urgent care space, OrthoFi in the orthodontics space, Experian not a ton of point solutions. Frankly a lot of times, the way that it's framed to me in the sales cycles that I'm involved in is they're considering either a point solution or they had the buyers, the CFOs, the VPs of RCM or they had a point solution that didn't work, and they're reticent to mention the name of the point solution because they want to get rid of it.

And so they find it immaterial to share with us. But I think that the way I would break it down is folks who look into those point solutions are either considering them in comparison to Thoughtful AI or **they've had a bad experience** or they are very forward thinking, and so they don't want a point solution. They want something that can operate their end-to-end process for a given RCM process.

Client 00:20:20

Got it. This is super helpful. Maybe just to circle back to prior auth, like just to outline what are the actual like path that what typical prior auth process is taking? And like what is the AI agent doing?

Expert 00:20:33

Yes. Sorry, just to clarify, just like the path of how like a given provider submits and monitors a prior auth and how an AI agent would do the same thing?

Client 00:20:42

Yes. If I were to do like a simple flow chart just step by step what's actually happening just like if you could walk through that.

Expert 00:20:50

Yes, totally. And I'll give you a general outline. This is going to, of course, be nuanced and may include fewer or, I'd say, additional steps depending on the specialty. But essentially, a patient has an appointment is registered for that appointment. They submit insurance information to the provider. And I'm going to be very pedantic. So just forgive me, I know you guys know this, but I know many of these steps.

So that insurance information prior to the services that are going to be delivered either because there's been a referral in some cases that this service needs to be delivered for this patient or because the patient has requested that service.

The RCM team at a given provider is going to do two things. One, they're going to verify that the patient does still have the insurance that they claim to have when they submitted their insurance card or filled out a form, they're going to look for the eligibility verification.

And then just in general for the insurance. And then for the specific services, whether that's via a referral or whether that is because the provider or the patient believes a certain service is needed, they will then check to see if, in addition to just general insurance, if a prior authorization from the insurance provider is needed.

Now what they will do is they will submit the service, they will look into the portal and they will submit the service and they will submit any documentation that would be needed in the form of medical records or a letter from a clinician, a referring clinician to that payer and then they will essentially wait until the payer provides approval for the prior authorization or denies the prior authorization either straight up or asks for additional documentation.

Now in an ideal world this is handled in a somewhat seamless way between the EHR or the practice management system and the payer portal. In a lot of cases that we encounter, it's a human being from the RCM team doing each of these steps and then following up, calling, checking a work queue of prior authorizations that haven't been approved yet, calling the payer or sending e-mails or clicking contact

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support for the insurance payer to get the information on whether the prior auth has been approved or not.

And then once that prior auth is approved, in the ideal sense, that either flows ideally seamlessly back into the EHR or the RCM employee physically takes documentation that the prior authorization has been approved, uploads it to the EHR so that the services can be delivered prior to the patient actually receiving those services.

Now that's very sort of high level in general. What I will say is we encounter a lot of situations where prior authorizations are not done in a timely fashion. And so there's almost like an emergency prior auth as the patient is directly waiting to receive the services at the provider. **Situations like that are tricky in the sense of that** you have to have somebody essentially call and physically make sure that, that prior auth is done right then and there.

But you can imagine, as long as there are technologies involved, systems and documentation that can be submitted and a way to track the status, et cetera, then an AI agent can do all of that in place of a human. But I'll stop there. That's very general. I'm just giving you what I've seen. And if you have specific specialties you're interested in, I can tell you what I've seen there as well.

Client 00:24:19

Got it. And maybe each of those steps, can you outline where or what the bot is doing, what AI agent is doing, whether that's like screen navigation or like using GenAI, it would be helpful just to think through the use case as well.

Expert 00:24:32

Totally, yes. I'll tie it to the technology. So an AI agent that is handling prior authorization for a provider will log into the EHR, whether via API or direct UI access. And will take a work queue of prior authorizations that need to -- registrations that require prior authorization. So for a given appointment in the EHR, they will take the patient registration. The AI agent will navigate the screen to see what are the services, who is the payer and then the AI agent will follow the workflow that a human would follow.

If there's some automated way of submitting that from EHR to payer, the AI agent will see what is the documentation. It's following rules. It's following a rule set. We'll see what documentation is required to be submitted. We'll find that documentation within the EHR or if the documentation is missing, we'll send a note via the EHR to the RCM staff or if it's to the patient and there's SMS or e-mail enabled can send a request for that documentation to the patient.

Once that documentation is received, then the agent will upload that to the format of the prior authorization packet, whether that's from the EHR or within the payer portal, so navigating to a different web-based payer portal there to using, of course, the unique identifier of the patient itself, himself, their self and actually submit that documentation in the format that the payer requests for the prior auth to be given.

So navigating the screens and then monitoring. So you think about the AI agent is following an SOP and is generally using screen navigation, simple OCR or RPA at the basic level. And then if, for instance, a prior authorization comes back and requires a letter to be submitted, either an appeal letter or maybe a letter was required at the beginning, the agent, based on the rule set that the agent is following, will take the notes that are in the HER.

And usually, there's a field, whether it's a free notes field or a specific place where documentation is uploaded in the EHR, we will take that information. And based on the rules that's been given, we can use GenAI to generate a letter in the format required for a given payer to then take that letter, add it to the prior authorization packet and either submit it initially or resubmit it if it's an appeal.

And then the AI agent will monitor the status via the EHR and/or depending on the workflow via the payer portal to see until that prior authorization is essentially resolved. So again, just recapping, logging in, screen navigation, picking up a work queue, following a set of rules to take documentation, attach it to a prior authorization packet, submit that prior authorization packet either within the EHR.

Or within a clearinghouse or within the payer portal itself, can log into the other systems, monitor the status based on the unique identifiers and then where GenAI may come in is if letters need to be generated from either clinical notes or RCM team notes to generate something that would then be submitted again.

So a lot there. Happy to answer clarifying questions as needed. But if you think about the technologies real quick, it's RPA, OCR and certain GenAI capabilities, either using LLMs or retrieval augmented generation to pull from sort of approved external sources.

Client 00:27:53

How do you deal with like constantly changing portals or like UI?

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Expert 00:27:57

Yes. So a lot of it, honestly, it's a nuanced answer. Depending on the nature of the workflow and how the provider wanted the AI agent to be architected, it could be handled in a number of ways. One, if the process was really built sort of following -- like the AI agent was built using RPA alone, if the agent runs into an exception, then our engineering team looks into the exception and will program the new rules for, let's say, a payer portal change into the agent.

That's sort of like the basic RPA example. If, for example, the workflow and how we architect it for a provider enables -- it allows the agent to tap into the LLMs or sort of the RAG elements of AI, then the agent will actually attempts to solve the exception it has run into flagging what might have changed in the payer portal.

And then we'll send a notification to the human RCM team so that the humans always are in the loop of what the agent thinks that it should do in a given situation. And then once that's approved, and there are various ways Thoughtful can set this up for a provider. Once that's approved by a human, then the AI agent will use its -- the learning it has done to continue down the process of handling whether that's button changes or policy changes or documentation changes within the payer portal.

But it's important to note that an AI agent is not allowed to make decisions fully on its own without human-in-the-loop approval just because, for instance we're dealing with sensitive PII and things like that. So a little bit there, and I'm happy to clarify or add additional details.

Client 00:29:33

Got it. How frequently do these agents like break or run into issues?

Expert 00:29:38

So I mean, I think what I can tell you is there are always going to be exceptions that an agent encounters. The goal of Thoughtful and what I have seen, and I think if you talk to -- if you look at some of our case studies, the publicly available information, our goal is always to get the number of exceptions, meaning in a given run, let's say, for eligibility verification or prior authorization or claim submission.

Our goal is always to get the percentage of exceptions that a human RCM team has to deal with down of the overall transactions processed by the AI agent. So there are almost always going to be exceptions, but **the most successful of the Thoughtful AI customers have been able to drastically reduce the percentage of work** their human teams have to do because the AI agent can handle most of transactions without needing breaking, if you want to call it that.

Client 00:30:32

Is that reduction like commonly achieved? Or what is kind of typical for this type of solution?

Expert 00:30:37

I just want to be mindful of what I can and cannot share from like a proprietary perspective here. So I can't answer that for you, but I think check out our case studies and just leave it at that.

Client 00:30:48

Yes, maybe not as really Thoughtful but as we just think about RPA bots in general.

Expert 00:30:54

Fair enough. Yes. I'm happy to answer that question. And I'll say that Thoughtful isn't simply RPA. But if **we think broadly defined, Gen one RPA bots in a bad situation**, they're probably handling 50% of transaction volume. And in a good situation, RPA bots alone are maybe handling 80% of transaction volume, if we're talking strictly Gen one RPA versus the AI agent, whether Thoughtful or not, an AI agent that actually leverages RPA in addition to some of the other AI technologies.

Client 00:31:24

Got it. So like industry would be like 50% to 80%?

Expert 00:31:28

Yes. I think that's a fair publicly available number in the health care provider space. Now I'll just add one thing. There are certainly going to be use cases if the use case is extremely simple, where even an RPA bot could do this perfectly every time.

If you're talking about a few steps, very limited system complexity and very little to no variation in terms of the documentation or information

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that an RPA bot is required to take and work through its process. But in health care, most of the processes, as you all well know, are pretty complex. So I'd say 50%, 80%.

Client 00:32:03

Got it. And so just to clarify, this is kind of like an AI-enabled RPA bot is something analogous to Thoughtful, but maybe not as solution?

Expert 00:32:12

Yes. I think that's fair. And honestly I'll be real with you. I give you wide ranges because I want to be both accurate and sharing publicly available information. You're undoubtedly going to find edge cases, both like of less than 50% and of greater than 80%. But those are numbers that when I talk to providers and if you ask CFOs and VPs of RCM, they commonly share. And as you get closer to the 80%, they get more and more excited.

Client 00:32:40

And then just in terms of kind of the buyer you know that you're going after, what was kind of typical customer, you said not really hospital systems. So like what does that leave in terms of your focus?

Expert 00:32:52

Yes, totally. So we target nonhospital providers that the buyers we look at are CEOs, CFOs and VPs of RCM primarily. We are generally looking at rapidly growing groups of practices. So think of practices with, let's say, $100 million or more in revenue, **they are trying to scale and grow** rapidly.

Some examples are like dental service organizations or orthodontic, like PT service organizations, think of PE-backed roll-ups, the buyer, again, from a seat perspective, CEO, CFO, VP of RCM and then traits of them are non-hospital groups that are rapidly scaling and are often PE-backed generally with revenue north of $100 million annually.

And the more an organization relies on a large human RCM team, the more likely Thoughtful or another provider, a solution provider like us would be successful. And then specialties, as I mentioned, you can see our case studies, but I'll just say them behavioral, dental, orthopt, vision, podiatry, vascular that we have done particularly well with those groups. And then I'm sure there are other providers maybe not exactly like us, but trying to solve similar challenges that handle other specialties very well.

So there have been some changes. All what I can certainly say with confidence is Thoughtful AI and other solutions providers like us typically tend to price based on the value and ROI we anticipate delivering in terms of several buckets.

One, the number of FTEs that a bot will replace or will allow to either be reduced or reallocated to other work. Number two, the dollar size of a backlog of claims that the AI agent will be able to work through very, very quickly and/or the amount of like tech spend that can be replaced with an AI agent. And then finally, like the outstanding revenue capture **that may not be happening now due to claims denial rates or AR follow-up**.

All of those elements together get built into a pricing model that when pitched to CFOs and VPs of RCM, both have sort of like a dollar value ROI, like three to 5x or whatever the number ends up being. But oftentimes, it is grounded in thinking like how many FTEs would you need to get the same ideal outcome and then see how small the price of what a solutions provider is charging you versus that FTE cost.

Client 00:35:11

Helpful. Maybe just two clarifying questions. One is like how you not necessarily like a number or like kind of formula, but more so like are you pricing a number of transactions? Is it per bot?

Expert 00:35:22

Yes. So per agent. So there is a cost per agent that is dependent on the complexity of the process and that's essentially like the -- we don't charge a per transaction cost. We don't charge per transaction cost. The cost that we end up charging is on an individual agent basis, if you will. So one agent would cost less than two agents, less than three.

And then the cost of a given agent may change depending on the complexity of the process at a different provider. So if you look at our website, for example, like the eligibility verification agent, EVA may cost the same at provider A and provider B or it may be different if the eligibility verification process and the systems involved are drastically different at provider A and provider B.

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Client 00:36:08

Cool. And then are like the number of agents tied to the volume of transactions, like is there like a limit per agent? Or is it more like each agent is doing a discrete and different like workflow?

Expert 00:36:20

The latter. It's entirely based on the workflow itself. So like the agents are handling a particular RCM workflow or process and the volume assuming you're not talking about an order of magnitude of like 10,000 to one million transactions is immaterial.

Client 00:36:36

Interesting. And then like would it be like one agent is capable of handling like the whole prior auth process, for example? Or would you have like a chain of like four or five agents doing different things to make.

Expert 00:36:48

It would be a single AI agent for the entire prior authorization workflow. The complexity of the agent and the associated cost would depend on how complex the prior auth process is at a given provider. But it would be a single agent in terms of how Thoughtful architects, how Thoughtful sales and also more importantly, how the provider experiences the outcomes and output.

Client 00:37:11

Cool. And then just in terms of like upper limit of the number you can sell to a customer, it seems like you kind of have -- like when you think about like eligibility and prior auth and maybe like claims processing, like what is the total number of agents.

Expert 00:37:27

Yes. So I mean, the simple answer is it could be as few as one or two agents. But if you look at the case study for Signature Dental, for example, on our website, Cara, the VP of RCM there, I want to say that Cara has 10 to 12 agents now because if you think about -- like what we have on our website is really like our main go-to-market focus.

However, Thoughtful, this is publicly available information. Like anything I shared with you, this is publicly available information. Thoughtful can and will build an AI agent for just about any use case that we deem technically feasible and where ROI would be delivered for the provider customer. Our typical entry point is almost always via RCM, and that's where we really focused our go-to-market efforts. But you could have an unlimited number of agents if you wanted, if you expand it out of RCM to finance to HR to other process areas as well.

Client 00:38:23

Maybe just the one last question I had is curious how receptive this buying universe has been to this type of solution because you can go to a lot of these groups and they're just going to fully outsource RCM.

And they're going to go to full stack or they're going to outsource like meaningful components to it and then have some meaningful components. This is like something perhaps even more light touch. And so you kind of need to find the perfect group that wants like not too much, but not too little. And so like how big is the buyer universe, but like how receptive was that sell kind of like qualitatively?

Expert 00:38:59

Yes. I mean **I've been shocked how receptive, honestly, the buying universe is**. And the reason -- I'll give you a couple of quick points on that. The AI agent, while the AI agent is doing the work, everything that the AI agent does is visible to the provider in our user interface in the form of the telemetric data, everything down to a given record and how it was run.

So while there may be a feeling that with your outsourced team, **you may be losing control with an AI agent**, the providers themselves control the AI agent even if the agent can be set up to run autonomously or on a certain schedule.

So all of what the AI agent is doing in any given eligibility verification run or prior authorization run is visible to the provider team via our user interface. So one, from a control and visibility perspective, it's usually a very easy sell. Two, from a cost perspective, we are usually, from what I've seen, usually less expensive than -- a given AI agent is usually less expensive than an outsourced team that may be handling a given process and set of transactions associated with that process.

And in general, just to give you an idea, like we have a ton of inbound interest and those conversations usually progress at least through exploration because the buyer universe, at least in my experience, is probably 75% interested in exploring more and then the 20% to 25% that isn't are either very old school RCM folks who really are not interested in technology.

Or those who are doing really well with some of the automated solutions like via Waystar, et cetera, because maybe they don't have

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complex processes and they've been able to get their EHR and their clearinghouse to do a lot of the work for them. But anyway, I hope that helps.

Client 00:40:47

And do you guys sell into like maybe billing company that wants to reduce their fully outsourcing on behalf of the client?

Expert 00:40:54

Some of those use cases where, let's say, like an AI agent would operate either a billing system or, let's say, operate the RCM services of like an EHR or a PMS provider have been explored. They're not the core use case, but I think it's fair to say that conversations have happened and will likely continue happening.

Client 00:41:13

Got it. And then curious if there are any other customer types you have like kind of receptivity with like I know pharma sometimes comes up in kind of the prior auth process, but just curious if there are any kind of others that are maybe not core but potential customers.

Expert 00:41:30

Got it. Yes. You mean in terms of areas where there's a lot of prior auth happening? Yes, med device, pharma, infusion, which infusion often, it can have some pharma element there. I would say those are the three big ones that **we come across that are maybe not core specialties, but where there is great potential and a lot of interest from the buyer universe**.

Client 00:41:52

And on my last question, I know we're running over on time, but can I just circle back to something you said about like in the scale pitch figuring out like total ROI and kind of substitution costs in terms of like like collections and all that. How do you like to develop that? Like does the customer give you access to their data before in order to kind of go through this exercise? Or like what is kind of that process look like?

Expert 00:42:19

You mean the overall process or a specific piece of figuring out ROI?

Client 00:42:23

Like the overall, I guess the customer what's the ROI?

Expert 00:42:26

I have found that as long as you're talking to the CFO or the VP of RCM and they're serious, they'll share those details. I ask directly, and they'll share them because usually, they are very interested usually in what Thoughtful AI can, in theory, provide.

And so once if they are interested, I'm very clear and the other sales leaders are very clear, like we want to help you build a business case for this. In order to do that, I need to know a little bit about these things. So I found that they're very forthcoming. But again, you need to be speaking to the right level of stakeholder, and it's a big signal of buyer intent if they're not willing to share that information.

Client 00:43:05

Got it. Well, I know we've run over time now, but this call has been great. So I really appreciate your help today. Thanks for taking the time, super helpful. Hope you have a great rest of your week.

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